A

CLIENT'S

FAMILY CENTERED MATERNITY CARE STUDY

ON

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PREFACE

Family centered maternity care is a systematic approach used in giving holistic care for a pregnant woman and her family through Pregnancy, Labour and Puerperium. It forms part of the requirement of the Nurses and Midwives council that every midwifery student undertakes to qualify as a registered midwife. This care is based on understanding the client as a unique individual with a specific problem or need that must be addressed. It is during this period that the student midwife applies all knowledge and skills acquired in midwifery lectures to care for the woman, her baby during the antenatal, labour and puerperium.

Also, the family centered maternity care study helps student midwives to use the new trends in midwifery to ensure proper management of pregnancy, labour and puerperium using the nursing procedure that is identification of problems. It also helps to put into practice safe motherhood initiative which has been adopted by Ministry Of Health in order to help reduce the maternal and infant mortality and improve quality health care throughout pregnancy labour and puerperium. This gives the student midwife the opportunity to give the right health education to the client and family.

Due to this the student midwife has the opportunity and responsibilities to function in order to promote health care using the knowledge and skill acquired during the three-year training programme.

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INTRODUCTION

Family centered maternity care study is a systematic and comprehensive obstetric care rendered to the expectant mother involving the entire family during pregnancy, labour and puerperium without any complications to both mother and baby. The family centered maternity care study was carried on Madam O.F, a 36-year-old pregnant woman, Gravida 3 para 2, alive during her pregnancy, labour and puerperium. I met Madam O.F on 10th of June, 2024. at Nsoatre Health Center when she was at 36 weeks of gestation, I met her at the OPD of the Health Center when she was complaining about her Haemoglobin level and heartburns. I approached her nicely and her name and where she lives were asked and viewing of her antenatal book she fell within the criteria. Her permission was sought to take her as a client so she could be nursed through pregnancy, labour and puerperium. She was introduced to the midwife in charge as a client to be used for my care study and permission was granted.

Madam O.F was cared for during antenatal period. Her home was visited to know her family, her surroundings and the community where she lives. She was given the required education especially on her nutrition, support and management through the four stages of labour, the management included support and encouragement during the period of pregnancy, labour and puerperium. Her Estimated Date of Delivery was 7th July, 2024.

Madam O.F came on admission on 28th of June, 2024 and delivered a bouncing baby boy at 9:40pm who weighed 3.3kg.

Detailed information is captured in various chapters of this script. There are four chapters in all.

- Chapter one deals with assessment of client consisting of personal and social history, family history, past medical and surgical history, menstrual history as well as past and present obstetric history.

- Chapter two talks about the first meeting with the client and various home visits.
- Chapter three emphasizes on admission and management of the stages of labour including examination of both mother and baby, problems identified and nursing care plan.
- Chapter four deals postnatal care rendered to the mother and the baby from the day of delivery to the end of the first week after delivery, the problems identified during this period and the nursing care plan on puerperium.
- Summary and conclusion, bibliography as well as appendixes like complete diagnostic investigation, pharmacology of drugs, antenatal records, labour and postnatal records as well as baby records are included in the script.

WHY I CHOSE MY CLIENT

Madam O.F was chosen as a client on 10th June, 2024 at Nsoatre Health Center which happened to be her 9th visit. I approached her because I saw how worried she was talking about her Haemoglobin level and how she was complaining of not able to eat well because of heartburns. I took the chance to go through her antenatal card and decision was made to take her as a client since she fell within the criteria. Permission was sought to take her as a client for the care study to educate and enlighten her throughout her pregnancy and the Midwife in charge was informed which she agreed to. On thorough examination from head to toe and during abdominal palpation; no abnormality was detected. She was 36 weeks of gestation. All the necessary particulars were collected from her for follow up. Phone numbers were exchanged and an appointment was made for home visits.

LITERATURE REVIEW

PREGNANCY

Tiran (2021) defines pregnancy as a process from conception to delivery of the foetus; normal duration is two hundred and eighty days (280 days, 40 weeks or 9 months and 7 days), counted from the first day of the last normal period to delivery, or two hundred and sixty-five (265), from conception to delivery.

Myles (2020) stated that, during pregnancy the uterus plays a remarkable role by stretching and expanding to accommodate and nurture the growing foetus. It further states that the expansion and activation take place in the middle layer of the uterine wall, the myometrium, which is partly covered by an outer layer of the peritoneum, the perimetrium. It also states that all changes that occur during pregnancy are due to hormones.

During pregnancy, oestrogen levels also rise. The raised progesterone and oestrogen levels also causes some changes in the muscle layer and the epithelium of the vagina such as thicker and viscous mucous helping to prevent ascending infection as well as enlargement of breast and other organs in order to support foetal development and growth.

According to Ghana Health Service (2016), the number of times a client needs to be seen during pregnancy may vary. For the uncomplicated pregnancy it is recommended that at least four ANC visit should be made according to the following schedule.

- First visit: from onset of pregnancy up to sixteen weeks (16) gestation.
- Second visit: from the 24th to 28th week of pregnancy
- Third visit: at 32nd week of pregnancy
- Fourth visit: at 36th week of pregnancy

CHANGES IN THE SYSTEMS

The Breasts

According to Myles (2020), due to the increased blood supply, and stimulation in secretion of oestrogen and progesterone from both corpus luteum and placenta, major changes take place in the breasts during pregnancy, and new ducts and acini cells are formed. The greatest activity takes place in the first trimester, but proliferation continues throughout pregnancy.

The Uterus

Myles (2020) states the following changes in the uterus:

Endometrium (Decidua): After embedding of the blastocyst there is thickening and increased vascularity of the lining of the uterus or the decidua. Decasualization, influenced by

progesterone and oestradiol, is most marked in the fundus and upper body of the uterus (the usual sites for implantation). The decidua and trophoblast also produce relaxing, which appears to promote myometrium relaxation, and may have a role to play in cervical ripening and rupture of foetal membranes.

Myometrium: In early pregnancy, uterine growth is due to hyperplasia (increase in number due to division) and hypertrophy (increase in the size) of myometrium cells under the influence of oestrogen. As gestation increases, hyperplasia is less important and hypertrophy accounts for most of the growth of the uterus. In the latter half of pregnancy, the uterus expands mechanically owing to distension of muscle cells by the growing foetus and placenta. The uterus increase in weight from $57 - 60g$ to $1000g$ at term and increases in size from $7 \cdot 5 \times 5 \times 2 \cdot 5cm$ to $30 \times 22 \cdot 5 \times 20cm$ at term. The myometrium is both contractile (can lengthen and shorten) and elastic (can enlarge and stretch) to accommodate the growing foetus and allow involution following the birth.

Perimetrium: The perimetrium is a thin layer of peritoneum that protects the uterus. It provides a relatively inelastic base upon which the myometrium develops tension to increase intrauterine pressure. It does not totally cover the uterus, being deflected over the bladder anteriorly to form the uterovesical pouch, and over the rectum posteriorly to form the pouch of Douglas. The double folds of the perimetrium (broad ligaments)**,** hanging from the uterine tubes and extending to the lateral walls of the pelvis, become longer and wider with increasing tension exerted on them as the uterus enlarges and rises out of the pelvis. The anterior and posterior folds open out so that they are no longer in apposition and can therefore accommodate the greatly enlarged uterine and ovarian arteries and veins. The round ligaments (contained within the hanging folds of the perimetrium) provides some anterior support for the enlarging uterus and undergo considerable hypertrophy and stretching during pregnancy, which may cause discomfort or strain.

The Cervix: During pregnancy, the cervix remains firmly closed providing a seal against external contamination and holding in the contents of the uterus. It remains $2 \cdot 5cm$ long throughout pregnancy but becomes softer and swollen under the influence of oestradiol and progesterone. Its increased vascularity makes it look bluish in colour. Oestradiol stimulates growth of the columnar epithelium of the cervical canal. Under the influence of progesterone, the mucous glands become distended and increase in complexity resulting in the secretion of a thick, viscous, mucoid discharge. It forms a cervical plug called the operculum, which provides a protection from ascending infection (Myles, 2020).

The Vagina: During pregnancy, the muscle layer hypertrophies and oestrogen causes the vaginal epithelium to become thicker and more vascular. Its violet colour is probably due to hyperaemia. The altered composition of the surrounding connective tissue increases the elasticity of the vagina making dilatation easier during the birth of the baby. In pregnancy, there is an increased rate of desquamation of the superficial vaginal mucosa cells. These epithelial cells release more glycogen, which is acted on by Doderlein‟s bacilli, a normal commensal of the vagina, producing lactic acid and hydrogen peroxide. This leads to the increased and more acidic (pH 4.5-5.0) white vaginal discharge known as leucorrhoea. While this provides an extra degree of protection against ascending infection by some organisms, others such as Candida albicans can more easily establish themselves, resulting in more frequent occurrence of vaginitis in pregnancy (Myles, 2020).

Myles, (2020) states certain minor disorders of pregnancy such as;

- **Ptyalism:** which is the excessive production of saliva throughout pregnancy. Its cause is unknown but progesterone and/or HCG may be responsible for the increased viscosity which reduces with advancing gestation. Gastric acid is thought also to affect the volume of saliva.
- **Pica:** it is the persistent craving and compulsive consumption of substances such as ice, clay, soap, coal or starch. The exact cause is unknown but it has been associated with the following, due to deficiency of iron or zinc or sensory enjoyment for the texture/smell of food.
- **Nausea and vomiting:** Symptoms usually begin in the 4th week of pregnancy with a marked increase between 5 and 10 weeks when HCG levels are at their highest, followed by a steady decline until 20weeks. It is associated with enlarged placental size with increased amounts of HCG that occurs in the female foetus, multiple pregnancy or hydatidiform mole.

LABOUR

The World Health Organisation (WHO) defines normal labour as one that is low risk throughout, spontaneous in onset with foetus presenting by vertex, culminating in the mother and infant being in good condition following birth.

Myles (2020) states that, traditionally, three stages of labour are described as the first, second and third stage, but this rather pedantic view, as labour is obviously a continuous process. It has also been acknowledged that there are more than three stages of labour, namely; the latent, active and transitional phase, and these not only encompass specific physical changes but should also account for emotional effect observed in women during this time.

According to Annamma Jacob (2021), labour is divided into four stages

1) The *first stage* is that of dilatation of cervix. It begins with regular rhythmic contractions (true labour) and is complete when the cervix is fully dilated and effaced.
2) The *second stage* is the stage of expulsion of the foetus. It begins with complete dilatation of the cervix through complete birth of the baby.
3) The *third stage* of labour is that of separation and expulsion of the placenta and membranes. It lasts from the birth of the baby through the delivery of placenta and membranes with contraction and retraction of uterus.
4) The *fourth stage* is the first 4 hours after delivery of the placenta. This is an arbitrary time during which, the vital signs must stabilize and any tendency for immediate haemorrhage must be controlled. This is a critical period for every mother. The Ghana Health Service (2016) mentioned specific objectives during labour and delivery and these include;

1) Proper management of the four stages of labour.
2) Early identification and proper management of complications.

According to Ghana Health Service (2016), partograph is the recommended tool for the World Health Organization (W.H.O), for monitoring, documenting and managing labour.

W.H.O also stated some importance of partograph;

1. Provide guidelines on when labour is no longer “normal” and on the management for situations.
2. Gives a complete picture of the condition of the mother, baby and labour progress.

Annamma Jacob (2021) Partograph is a record of all the observations made on a woman in labour; the central feature of which is the graphic recording of the dilatation of the cervix as assessed by vaginal examination

Myles (2020) active management of the third stage of labour (AMSTL): An active management policy usually includes routine prophylactic administration of a uterotonic agent, either intravenously, intramuscularly or (occasionally) orally, as a precautionary measure aimed at reducing the risk of post-partum haemorrhage. It is applied regardless of the assessed obstetric risk status of the woman, and is usually undertaken in conjunction with clamping of the umbilical cord shortly after birth of the baby and delivery of the placenta by use of controlled cord traction.

From the above information, labour is when the uterus empties its content after 37 to 43 completed weeks.

PUERPERIUM

Myles (2020) stated that puerperium starts immediately after birth of placenta and membranes and continues for 6 weeks.

Ghana Health Service (2016), define post-natal period (puerperium) as the period from delivery to six weeks after delivery. The Ghana Health Service further stated the purpose of post-natal care in the clinic that, is to maintain the physical and psychological wellbeing of the mother and child. Care rendered during this period includes; 1. Inspecting pad for bleeding and lochia, its smell and consistency

2. Giving education on family planning.
3. Inspection of vulva and perineum for tear, swelling and pus.

Myles (2020) states that, following the birth of the baby and expulsion of the placenta, the mother enters a period of physical and psychological recuperation. The puerperium, starts immediately after delivery of the placenta and membranes and continues for 6 weeks. The overall expectation is that by 6 weeks after the birth all the systems in the woman‟s body will have recovery from the effects of pregnancy and returned to their nonpregnant state. However, it is only comparatively recently that there has been any professional recognition or substantial interest in the diversity and extent of the morbidity experienced by women in the weeks after childbirth. Some women continue to experience problems related to childbirth that extend well beyond the 6-week period defined as the puerperium, and the possibility of longer duration is now accepted alongside the range of initial morbidity.

It has been customary to refer to the first weeks after the birth as the postnatal period; defined in the UK as a period after the end of labour during which the attendance of a midwife upon a

woman and baby is required, being not less than 10 days and for such longer period as the midwife considers necessary.

By no longer stating an endpoint in the time when midwifery care can still be made available to women, it is to be hoped that offering more flexibility to the provision of midwifery care will in time also make a positive difference to the experiences of women and midwives of this aspect of midwifery care and services.

According to Veralls (2017) third edition puerperium is a period of six weeks following child birth during which the maternal uterus and other organs and structure are returning to their pre-pregnant state, lactation should be established and new infant should be accepted in to the family.

Tiran (2021) defines puerperium as the six to eight-week period following child birth during which the uterus and others organs and structures are returning to the non- pregnant state.

The woman recovers from the stress of pregnancy and labour. Mothers and baby are discharged after twenty-four hours (24 hours) that is when they are in good health. Mother assumes responsibilities for the care of the new born baby. Lactation and breast feeding are established, creating bondage between mother and baby.

During this period, the uterus involutes and traumatized areas of the genital tract heal. The care of the mother and the baby that is postnatal care is focused on the promotion of the physical wellbeing of the mother and baby through their normal regular postnatal visits. It also focuses on encouraging breastfeeding and promoting the development of good mother relationship. Throughout this period, the midwife encourages, supports and strengthens the mother's confidence in her thus enabling her to fulfil her role as a mother in the family. (Myles 2020).

Initiation of breastfeeding is established, vitamin A supplement is given to the mother, the baby is immunized with Bacillus Calmette Guerin (BCG) and Polio O vaccine by a community health nurse, vital signs such as blood pressure, temperature, pulse and respiration are checked and recorded. Any abnormalities are reported to the in-charge.

Puerperium can also be defined as the period following childbirth

during which the body tissues, especially the pelvic organs, revert

approximately to the pre-pregnant state both anatomically and

physiologically (Annamma Jacob 4th edition, 2015).

CHAPTER ONE
ASSESSMENT OF THE CLIENT.

1.0 INTRODUCTION

Assessment is the systematic process of gathering, analyzing and interpreting information or data to evaluate a person's knowledge, skills, behavior, health or other attributes. This chapter entails the client's profile or social history, lifestyle or hobbies, home environment, family history, medical history, surgical history, menstrual history, past obstetric history and present obstetric history.

1.1CLIENT'S PROFILE/SOCIAL HISTORY.

Madam O.F aged 36, gravida 3 para 2 all alive was born on 7th November, 1988. She is an

Akan who comes from Sunyani in the Bono Region and stays at Nsoatre in the Bono Region. She has a height of 158cm and a weight of 66kg. She is dark in complexion. According to my client, she had her education up to Junior High level. She is a trader who sells vegetables. Madam O.F says she can speak Twi fluently but she is ineloquent in English. She is married to Mr. G.A, who is 37 years of age. Mr. G.A also ended his education at the Junior High level and he is a carpenter as stated by my client. According to my client, she has two daughters. The first one is 10 years and the second one is 6 years old. My client says her husband is her next of kin and she doesn't drink or smoke.

1.2 CLIENT'S LIESTYLE/HOBBIES.

According to Madam O.F, she wakes up at 5am every morning. She passes stool every morning and sometimes in the evening. She mostly passes stool once every day. She stated that they visit the public toilet when they want to ease themselves. After easing herself, she brushes her teeth with toothbrush and toothpaste and makes breakfast for herself, her husband and children. She mostly makes corn porridge or rice porridge with bread but she sometimes makes tea that is milo with milk powder and bread when her children are late for school. After she is done making the breakfast, she boils water for her children. Based on my client, their source of water

is from the community pipe borne water. Her daughters fetch water and they store the water in barrels. She wakes her daughters up around 6am. According to her, her first daughter helps her with most of the chores. She sweeps the compound and disposes off the refuse in a covered dustbin and empty them in a dumping bin few meters away from the house whiles her second daughter takes her bath and have breakfast. The first daughter takes her bath after she is done with her house chores. She then takes her breakfast and leave to school around 7:30am together with her kid sister. Madam O.F stated that, she gets ready to go and sell her vegetables by 8am so her husband is the last person to leave the house. Madam O.F stated that she sells her vegetebles at the Nsoatre market and it is not very far from her house. She also stated that, during the afternoon, she eats fried yam or roasted plantain with groundnut. Her children close from school at 3:30pm and they come to where she sells her vegetables. By 5pm, they start packing and get home around 5:30pm. According to Madam O.F, she makes dinner immediately she gets home. She eats all types of foods but she likes eating rice and tomato stew most. Her husband gets home around 7pm and have his dinner. According to Madam O.F, watching television is her hobby and she mostly watches it when she has nothing to do so after having dinner and taking her bath, she switches on the television and watches it with her children and husband. At 9:00pm, she goes to bed. Since my client and husband are Adventists, their weekend starts from 6pm on Friday. As stated by my client, on Friday, she closes from work at 5:00pm so that she can prepare her dinner early. She saves some of the food she cooked for dinner for Saturday morning. On Saturday morning, she takes her bath, eat the previous meal with her husband and children and get ready for church. They mostly go to church around 9:00am and close at 12:00pm. She mostly cooks braised rice in the afternoon after church. She usually prepares fufu with light soup or ampesi with garden eggs stew during supper. Because she is an Adventist, she does her washing on Sundays.

1.3 CLIENT'S HOME ENVIRONMENT

PHYSICAL

According to my client and what I saw during home visit, she lives in a green painted house built with cement blocks and roofed with aluminium roofing sheets. There are two rooms in the house, she sleeps in one with her husband and the other one is occupied by their two daughters. She has a kitchen on her compound built with woods as well as bathroom built with cement blocks on the compound. She used to pack luggage in front of their wooden window so she was not getting enough ventilation but I ensured her to pack the luggage away from the window to get enough ventilation when I went for an antenatal home visit.

Madam O.F sleeps on a firm mattress and she sleeps in a treated mosquito net. The environment was very clean but I noticed the area around her kitchen was bushy but she got someone to clear the weeds. Madam O.F disposes off her refuse in a covered dustbin which her daughters empty every morning. She verbalized that their source of water is from the community pipe borne water. She stores her water in a clean barrel with a lid covered on the barrels and their source of light is from the electricity and they had a good drainage system.

PSYCHOSOCIAL

According to Madam O.F, she lives in harmony with her neighbours as well as the community members which gives her peace of mind and she has never quarreled with anyone before and even during pregnancy. She stated that she has good interpersonal relationship with other family members and they help her with the chores when they come around.

According to her, she is prepared mentally for the pregnancy together with her family.

1.4 CLIENT'S FAMILY HISTORY

Madam O.F is the first born out of four children of Mr. K.C and Madam A.C from Sunyani in the Bono Region of Ghana. She has three siblings who are boys. According to Madam O.F, her mother is a hairdresser and her father is a farmer. They do not have any hereditary disease like Hypertension, Diabetes, Heart Disease, Asthma, Sickle Cell Disease, Mental illness and birth defects. There is no history of multiple gestation and according to her, there is no blood related illness like leukemia, sickle cell anaemia and others and death in the family occurs naturally. They are all Christians.

1.5 CLIENT'S MEDICAL HISTORY.

According to Madam O.F, she has never been admitted to the hospital unless she is going to deliver. She has never been treated of any serious medical disease like Hypertension, Diabetes, Epilepsy, TB, Kidney Disease, Sickle Cell Disease or Psychiatric condition. She has had no blood transfusion. She has no food and drug allergy.

1.6CLIENT'S SURGICAL HISTORY.

Madam O.F stated herself that she has had no pelvic fracture or undergone any surgery like hysterectomy, myomectomy, mastectomy, salpingectomy, caesarean section or any other form of surgery. According to her she has had no road traffic accident. No episiotomy was given during previous deliveries.

1.7CLIENT'S MENSTRUAL HISTORY.

Madam O.F had her menarche at the age of 10 years and had a regular menstrual flow all her periods except when she becomes pregnant. She has verbalized that she doesn't know her cycle but she normally bleeds for 5 days. She experiences dysmenorrhea but she does not experience menorrhagia during her menstruation. She wears one pad in the morning and one during the evening.

She resumes her menses about four months after delivery. According to her, her last menstrual period was 30th September 2023 using this information gives her Expected Date of Delivery as 7th July, 2024 and her Expected date of delivery according to scan was 6th July,2024.

1.8CLIENT'S PAST OBSTETRIC HISTORY-PREGNANCY

Madam O.F gravida 3 para 2 all alive, had her first pregnancy in 2013. According to her, she was a regular attendant of antenatal care clinic at Nsoatre Health Centre. She took one tetanus injection and all the five sulfadoxine pyrimethamine during her first pregnancy. Madam O.F stated that the pregnancy was carried to term without any complications like ectopic pregnancy or placenta abruption. She encountered minor disorders like constipation, backache, frequency of micturition and insomnia but they were all resolved after delivery and the manageable ones were managed. Madam O.F was counselled and tested HIV/AIDS and syphilis and proved negative and her second pregnancy was in 2017. She stated that she was a regular attendant at the Nsoatre Health Centre antennal care clinic. She verbalized that she took two tetanus and the three sulfadoxine pyrimethamine during her second pregnancy. She also stated that she had no complications. She encountered minor disorders such as ptyalism, constipation, backache and frequency of micturition but they were managed by the midwife.

LABOUR

Madam O.F said she had a spontaneous vaginal delivery on 2nd July 2014 where labour started around 4am at Nsoatre Health Center without any complications like postpartum haemorrhage. She delivered a live female and her weight was 3.0kg at birth. She cried immediately after birth and was breastfed after delivery. She was in good condition as stated by my client. Placenta membranes were completely delivered and perineum was intact.

The second child was born on 22nd August, 2018. According to Madam O.F, she had a spontaneous delivery at Nsoatre Health Centre without any complications. She delivered a live female and she weighed 3.1kg at birth, she cried immediately after birth. She was in good

condition and was also breastfed after delivery. According to Madam O.F, the placenta and membranes were all delivered and she had no postpartum haemorrhage and perineum was intact.

PUERPERIUM

Madam O.F said after her first delivery she was healthy during puerperium. She did not encounter any problems during breastfeeding. Breast was Lactating.

She breastfed the first born exclusively for six months after which she introduced complementary feeds like porridge, mashed kenkey and gradually to solid foods till she weaned her off at 2 years. She stated that she attended postnatal at Nsoatre Health Centre and the baby was immunized against diseases like; tetanus, poliomyelitis, tuberculosis, whooping cough, pertussis, hepatitis B, haemophilia, measles influenza B and yellow fever. Her first daughter did not experience any ill health. She did not experience any Secondary Postpartum Haemorrhage or puerperal blues. Her husband always supported and cared for her and the babies which enabled her to have enough rest and sleep. My client together with her husband decided to space out the children with family planning and the commodity used was progestin only pills which she classified as 28 tablets.

According to Madam O.F, she was healthy after her second delivery and breast was lactating. She breastfed her second born exclusively for six months and she introduced complementary feeds. She attended post-natal at Nsoatre Health Centre and baby was immunized. Her second daughter did not experience any ill health and she did not experience any secondary postpartum haemorrhage. She had enough rest and sleep. The family planning method used was progestin only pills.

1.9 CLIENT'S PRESENT OBSTETRIC HISTOY

Madam O.F Gravida 3 Para 2, all alive attended her first antenatal clinic at Nsoatre Health Centre on 10th November, 2023. When she was five (5) weeks pregnant. According to her, she could not remember her last menstrual period and according to the ultrasound scan results, her Expected Date of Delivery was 7th July, 2024. Her past obstetric, medical, surgical, family and menstrual history were eligibly recorded in her maternal record book.

Her baseline vital signs and laboratory investigations were done and recorded as follows;

Height	158 centimeters
Weight	66 kilograms
Blood Pressure	110/64 millimeter of mercury
Temperature	36.4 degree Celsius
Pulse	75 beats per minute
Respiration	20cycles per minute
Haemoglobin	10.8 grams per decilitre
Blood group	O
Rhesus factor	Positive
Veneral Disease Research Laboratory	Negative
Sickling	Negative
Hepatitis B Antigen	Negative

Urine protein/sugar negative/negative

Head to toe examination was done and no abnormalities were detected. Abdominal examination was done and on inspection, there was no scar or straie gravidarum but there was linea nigra present. She was physically examined from head to toe and no abnormality was detected. An abdominal examination and palpation were done and recorded as follows;

Gestation 5 weeks

Fundal height Not palpable

Foetal heart beat Absent

Presentation Not palpable

She was served with the following routine medications after educating her on nutrition and hygiene.

Tablet folic acid 5mg daily x 30 days

Tablet ferrous sulphate 200mg daily x 30 days

Tablet multivitamin 200mg daily x 30 days

I later asked my client her expectation with this pregnancy and she said she wish to have a baby boy since all her two children are girls.

CHAPTER TWO
ANTENATAL CARE

2.0 INTRODUCTION

Antenatal Care is the care given to a pregnant woman from the time conception is confirmed until the beginning of labour (Magaret Myles 17th edition).

This chapter deals with the first interaction with the client during antenatal period, subsequent antenatal visit to the clinic, first home visit and subsequent home visit as well as nursing care plan for the problems identified during the antenatal period.

2.1 FIRST CONTACT WITH THE CLIENT.

Madam O.F was a regular antenatal clinic attendant until I met her on the 10th of June, 2024 at 9:30 am when she was 36 weeks pregnant and had come for her ninth (9th) visit. She was seen at the Out Patient Department where she was going for her folder, I was passing and I overheard her say the midwives always tell her that her haemoglobin level is low which I saw on her face that she is worried, I approached her and established rapport, I later got the chance to glance through her antenatal book and realized her haemoglobin was 10.8g/dl, I asked if that is the only bothering her because her expression shows that she is going through pain and she said she has not been able to eat well for the past few days because of heartburns and I told her they can all be managed so she should not worry but apart from her haemoglobin level she had good obstetrical history so I decided to take her as my client for the family centered maternity care study in order to educate her more on what she can do to increase the haemoglobin level and also cope with heartburns. Madam O.F was happy about it and I also expressed my gratitude and then informed me in-charge about Madam O.F seeking her approval which she asked to her antenatal book and Madam O.F herself. I then asked for the contact of madam O.F which she gave to me and I helped her save mine too on her phone, I also told her I would be paying her visits to her house which she agreed.

I went further to explain to her how the procedure was going to be done and thanked her.

Her vital signs were taken and recorded as follows;

Blood pressure	100/60 millimeter per mercury
Temperature	36.3 degree Celsius
Pulse	80 beats per minute
Weight	74 kilograms

After recording her vital signs, I took her urine for sugar and protein and the results were negative. She was encouraged to take her routine drugs as ordered and keep eating well for herself and the baby in utero.

I reassured her that, she was going to have a safe delivery as she always comes to the clinic for examination. It was explained to her that the general head to toe examination was going to be carried out on her in order to assess her health and that of the unborn baby. I asked her to empty the bladder after which privacy was provided as she did so. Madam O.F was helped to lie in the dorsal position and then asked to lie in a lateral position. I washed, dried and rubbed my palms to warm them.

PHYSICAL EXAMINATION

On physical examination, her hair was neatly washed. There were no discharge from the eyes and the conjunctiva was pink with no signs of jaundice. Her ears and nose were not discharging and were situated normally and of normal size. Her gums were not bleeding and she had clean teeth without halitosis. The neck had no distended veins, no enlarged lymph node or goitre. I examined the chest and breast, and the chest was normal with good rise and fall which indicated good respiration.

BREAST EXAMINATION

Both breasts were exposed and inspected for the size and shape in which the sizes were normal and had hemispherical shape and they were heavy. There were no dimples or cracks on the nipples. The breasts were not discharging and were normal in length. The skin around the breast was smooth and had no rashes. After the inspection, one breast was exposed to be examined and the other breast was covered. Client was told to put the hand at the part to be palpated under the head. With the non-dominant hand supporting the breast, the dominant hand was used to palpate the breast systematically in a circular manner using the inner aspect of the fingers to find out if there were masses but on palpation, there were no lumps neither did she complain of any pain in the breast. The nipple was squeezed with cotton wool and fluid examined for its colour but no abnormality was detected. Same procedure was performed on the other breast and no abnormality was detected. After the breast palpation, I taught her how to examine her own breast and then care for the breast to prevent breast infection. I proceeded to the extremities starting from the upper extremities, Madam O.F was asked if she feels any tingling sensation and tightness of her fingers on making a fist and the response was no. Her hands and fingers were inspected for oedema and pallor of the palms and fingers which proved to be normal. The fingers were normal with short nails and her hands were equal. Her back was inspected for abnormalities like oedema of the sacral region, curvature of the spine but no abnormality was seen. On examination of the legs, I found out the legs were of the same length, no varicose veins were found, no oedema and no tenderness in the calf veins and no abnormalities were found, the toe nails were also short.

ABDOMINAL EXAMINATION

On abdominal inspection, it was ovoid in shape with normal size compared with her gestational age, and there was no scar, no striae gravidarum but linea nigra was present. Foetal movements were observed. I rubbed my hands together to generate heat before palpating the abdomen to

prevent inducing contractions. I then stood at the right side of Madam O.F and faced her. On fundal the upper pole of the uterus was firm, smooth and soft indicating that it was occupied with foetal buttocks.

The symphysio fundal height was measured from the symphysis pubis to the curved upper border of the fundus and it was 34 centimetres with 36 weeks gestation. The lie was longitudinal with cephalic presentation.

On lateral palpation

Still facing Madam O.F, I placed my hands on the left side of the uterus at the level of the umbilicus and gentle pressure was applied with my left hand by the side of the abdomen to hold in apposition. I palpated the abdomen gently with my right hand using a rotatory movement to locate the back which is smooth and continuous and the limbs which are irregular were located at the other end of the abdomen. The position was left occipito anterior.

On auscultation, the foetal heart beat was heard a little below the umbilical area of the mother's abdomen with good volume and rhythm. The foetal heart rate was 142 beats per minute.

DESCENT

I turned and faced madam O.F feet and I asked her to bend her legs in order to relax the abdominal muscles which she did. I grasped the side of the uterus just below the level of the umbilicus between the palms of my hands making sure I caused her no discomfort. It was hard and round indicating the foetal head was at the lower pole. With the descent of the foetal head, the anterior shoulder and the symphysis pubis was located, five fingers admitted between the anterior shoulder and the symphysis pubis which indicated that descent was $5/5^{th}$ above the pelvic brim. The lie was longitudinal and presentation was cephalic.

VULVA INSPECTION

She granted me permission when I asked to inspect the perineum and it was nicely shaved, there were no offensive discharge, sore, vulva warts, vulva varicosity and bleeding. After the examination, the client was helped to dress up, lie on her side before getting up from the bed. Madam O.F was thanked for her co-operation and made comfortable on a chair. The findings were recorded and communicated to her. She was encouraged to ask questions for clarifications. I again educated her on the signs of labour that is, blood stained mucoid discharge, frequent painful uterine contractions and also severe lower abdominal pain and backache and I asked her to report to the hospital immediately she sees these signs for delivery. Education on birth preparedness and means to reach the hospital when labour starts were given. I told Madam O.F to gather the items she would need for delivery into a bag. I also educated her on the proper use of the insecticide treated bed net. She was encouraged to ask questions for clarifications and she complained of backache. She was told it is because of physiological changes that occur in pregnancy due to weight of pregnant uterus and will resolve after delivery, I told her to avoid lifting heavy objects and wear low heel shoes. I finally emphasized more on the need to take the routine drugs, I educated her more on her nutrition, I asked her to eat foods rich in iron such as more green leafy vegetables and also not to overcook them before she eats, fish, beans, eggs, groundnut and others. She then asked if she could take malt and milk mixed together which I agreed on, after which I recorded the findings in the maternal health record book. She was served with routine drugs such as

Ferrous sulphate tablet 200mg daily x 7days

Folic acid tablet 5mg daily x 7days

Multivitamin tablet 200mg daily x 7days

I thanked her for her co-operation and expressed my desire to visit her at home which she agreed and I informed me in charge. I also told her she should report to the clinic if she feels unwell before the set date.

2.2 FIRST ANTENATAL HOME VISIT

On 12th June 2024, I visited Madam O.F as we agreed on, the purpose for the visit was to involve the entire family in her care, assess the home situation as well as find out about her health condition.

Upon reaching the house at 11:00am, Madam O.F offered me a chair to sit and offered me water, welcomed me and introduced me to the family members present. I quickly observed the environment and on observation, Madam O.F lives in a green painted house built with cement blocks and roofed with aluminium sheet. The house comprises of two rooms, there was a kitchen made of woods on the compound, there was also a bathroom built with cement blocks on the compound. There were electricity and the source of water for the family is pipe borne in the community. I asked about the health condition of the family and she said they were all doing well. Madam O.F stated that she lives together with her husband and two children live in the rented house on its own with a big compound. She stated that her husband mostly comes home late because of his work. I asked of her children and she told me they are in school. I sought permission to enter her room and permission was granted. I entered her room and I noticed the room was well swept but there were luggage packed on a table in front of her wooden window. I asked her if she gets enough ventilation and she said she uses one of her cloth to fan herself during the night. I told her that she should let her children pack the luggage away from the windows so that she can get enough air from the window and she agreed. I found out that she sleeps under treated mosquito net.

Madam O.F disposes off her refuse in a covered dustbin which her first daughter empties it every morning in the public dumping area few meters away from away from their house. I enquired her preparedness towards labour and asked her to show me what she had bought so far. Items such as baby's napkins, baby cloths, perineal pads and others were neatly packed in the bag.

Upon asking for support person, she said her first daughter is there to support her and her husband also helps whenever he comes home. At her home, Madam O.F had no complains. I thanked her for her co-operation and encouraged her to call me for support anytime and finally asked to leave.

2.3 SUBSEQUENT ANTENATAL HOME VISIT

On the 15th of June 2024, I visited Madam O.F again around 8:30am, the aim of the visit was to find out how she was doing and also whether she followed the education given to her about having enough ventilation. She offered me a seat and I asked of her well and that of her family too. On observation, the compound was well swept but beside the kitchen was bushy. I explained to her that bushy areas habor disease-carrying pests such as mosquitoes increasing the risk of malaria. I told her to get someone to clear the weed and she agreed. I sought permission to enter her room and check whether she packed the luggage away from the window. Permission was granted and I entered the room, I found out she has packed her luggage away from the window and she also verbalized she gets enough ventilation. I asked if she has any complaints and she complained of frequency of micturition during the night and backache. I reassured her that the frequency of micturition and backache can be managed but will be resolved after delivery and that is a physiological change as a result of the gravid uterus on the bladder and the backache is as a result of weight on pregnant uterus.. I advised her to reduce fluid intake before going to bed to reduce the frequency of micturition at night and also place a pillow between her knees when sleeping to support her lower back. I reminded her of

her next visit which was 17th June 2024. I thanked her for her co-operation and finally asked to leave.

On the 20th of June 2024, Madam O.F was visited again at 9:30am.The purpose of the visit was to find out how she was doing and also check whether she has cleared the weeds. She offered me a seat and I asked her of her well-being and that of the family. I observed the environment was very neat. I got closer to the kitchen and saw that the bushy area has been cleared and I applauded her. I asked if she had any complains and she verbalized she has no complaints.

.I thanked her for her co-operation and encouraged her to call me for support anytime she needs and finally asked to leave.

On 26th June, 2024, I went to visit my client again to see how prepared she is towards labour. I got there around 9:00am. The compound was neat as usual. She offered me a seat. I asked of her health and that of the family and she said they are all doing well. I enquired her preparedness towards labour and asked her to show me what she had bought so far and everything was intact. I helped her pack the items and including her antenatal card and told her to replace it anytime she removes it. I got the chance to meet her husband and I told him that her time is almost due so she shouldn't be left alone. Madam O.F complained that she was experiencing vaginal discharges and an explanation was given to her that it was as a result of the hormones such as oestrogen and progesterone released during pregnancy which softens the cervix. I encouraged her to report to the clinic if the discharge becomes offensive.

I encouraged her not to insert her hands or wash with any concoctions and also she should change her panties regularly or use perineal pads if discharge is too much. I thanked her for her cooperation and left.

2.3 SUBSEQUENT VISIT TO THE ANTENATAL CLINIC

On 17th June 2024, Madam O.F visited the clinic as scheduled around 9:47 am.

She was warmly welcomed. Her vital signs were checked and recorded as;

Blood Pressure	112/67 millimetre of mercury
Pulse	98 beats per minute
Respiration	20 cycles per minute
Weight	74 kilograms
Temperature	36.2 degrees Celsius

Specimen of mid-stream urine was collected and was tested for sugar and protein and the result was trace and negative respectively which I encouraged her to take in more fluids. Her haemoglobin level was checked and it was 12.4 grams per decilitre, she was praised for the improvement. Every procedure was explained to her understanding. She was sent into the palpation room and was asked to sit and lie on her side on the bed. I washed and dried my hands, privacy was provided. The head, nose, eyes and ears were inspected but no abnormalities were seen. The neck, breast, upper and lower extremities were examined for abnormalities but none was seen. Client's back was examined for oedema and also spine for abnormalities but none was detected. On palpation, foetal movement was felt, lie was longitudinal, presentation was cephalic and descent was 5/5th. The Symphysio fundal height measured 35 centimetres with 37 weeks gestation.

On auscultation, the foetal heart beat was 144 beats per minutes with a good rhythm.

All findings were communicated and recorded. Madam O.F complained of loss of appetite and I encourage her to eat bit by bit and take in more fluids and vegetables. She was asked to report

to the facility in a week time which was 24th June, 2024. She was encouraged to still adhere to the education given to her on her haemoglobin level since her time was almost due. I asked her about her previous complains which she stated that things are getting well. I educated her on the signs of true labour which included persistent contractions, severe waist pain, backache and a slimmy red discharge. I thanked her and saw her off.

2.5 NURSING CARE PLAN DURING ANTENATAL PERIOD

Nursing care plan during antenatal period involves identifying various problems during your care for a client and finding suitable ways of solving problems within a particular period holistically.

DATE	PROBLEMS IDENTIFIED
10/06/2024	Client complained of heartburns.
12/06/2024	Client complained of backache.
12/06/2024	Client complained of frequency of micturition.
17/06/2024	Client complained of loss of appetite
26/06/2024	Client complained of leucorrhoea.

SHORT TERM OBJECTIVES

Client will be relieved of heartburns within 72 hours

Client will be able to cope with reduced backache within 24 hours.

Client will be able to cope with frequency of micturition within 24 hours.

Client will be able to eat any food of her choice within 24hours

Client will adjust to leucorrhoea within 24 hours.

LONG TERM OBJECTIVE

Madam O.F will go through the pregnancy successfully and have a healthy live baby after delivery without any complications to her baby and herself.

NURSING CARE PLAN DURING ANTENTAL

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVE/ CRITERIA OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
10/06/2024 At 9:30am	Impaired body comfort (heart burns) related to relaxation of cardiac sphincter by progesterone.	Client will be relieved of heartburns within 72 hours as evidenced by the client verbalizing she has been relieved of the heatburns.	1.Reassure client 2.Explain the physiology of heartburns to the client 3.Advise client to avoid bending down while carrying out household chores 4.Educate client to avoid oily and spicy food 5 5.Educate client to eat small meal but at frequent intervals 6.Counsel client to sit up at least 30 minutes after eating or before lying down.	1. Client was reassured 2. Physiology of heart burns was explained to the client 3. Client was advised to avoid bending down while carrying out household chores 4. Client was educated to avoid oily and spicy food 5 5. Client was educated to eat small meal but at frequent interval 6. Client was counselled to sit up at least 30 minutes after eating or before lying down	Goal fully met as evidenced by client verbalizing that she is been relieved of heart burns. 13/06/2024 at 9:30am L.A

NURSING CARE PLAN DURING ANTENTAL

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVE/ CRITERIA OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
12/06/2024 at 11:05 am	Altered bladder function (frequency of micturition) related to pressure of foetal head on bladder.	Client will be able to cope with frequency of micturition within 24 hours as evidence by client verbalizing she is able to cope with frequency of micturition.	Reassure client that her micturition will return to normal after delivery. Explain the cause of the frequency of micturition to client 3. Encourage client to have a warm bath before going to bed. 4. Encourage client to reduce fluid intake before going to bed. 5. Encourage client to void before going to bed.	1. Client was reassured that her micturition will return to normal after delivery. 2. Client was educated on the cause of frequency of micturition. 3. Client was encouraged to have a warm bath before going to bed. 4. Client was encouraged to reduce fluid intake before going to bed. 5. Client was encouraged to void before going to bed.	Goal fully met as evidenced by client verbalizing that she is coping with the frequency of micturition on 13/06/2024 at 11:10 am L.A

NURSING CARE PLAN DURING ANTENTAL

DATE/TIME	NURSING DIAGNOSIS	NURSING OBJECTIVE/ CRITERIA OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
12/06/2024 at 11:10 am	Altered body comfort (backache) related to weight of pregnant uterus and altered posture in pregnancy	Client will cope with reduced backache in her pregnancy in 24 hours as evidenced by the client verbalizing that she is coping with reduced backache.	1. Reassure client. 2. Explain the physiology of backache in pregnancy to client. 3. Educate client to avoid lifting heavy objects. 4. Encourage client to have enough rest during the day. 5. Encourage client to wear low heel shoes.	1.Client was reassured 2. Physiology of backache in pregnancy was explained to client. 3. Client was educated not to lift heavy objects. 4. Client was encouraged to have enough rest during the day. 5. Client was encouraged to wear low heel shoes.	Goal fully met as client verbalized that she is coped with reduced backache 13/06/2024 at 10:00pm L.A

NURSING CARE PLAN DURING ANTENTAL

DATE/TIME	NURSING DIAGNOSIS	NURSING OBJECTIVE/ CRITERIA OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
17/06/2024 at 10:30am	Altered nutrition pattern (less than the body requirement) related to loss of appetite	Client will be able to regain her appetite within 48 hours as evidenced by client verbalizing that she is able to eat well	1.Educate client on the importance of food and its benefits during pregnancy 2. Encourage client to take fruits and vegetables and more fluids to prevent dehydration. 3. Educate client on oral care before and after eating. 4. Educate client to eat appetizing meals. 5.Administer multivitamin as prescribed	1 Client was educated on the importance and food benefits during pregnancy 2. Client was encouraged to take fruits and vegetables and more fluids to prevent dehydration 3. Client was educated on oral care before and after eating 4. Client was educated to eat appetizing meals. 5. Serve prescribed multivitamin E.g. Tablet B Complex.	Goal fully met as evidenced by client verbalizing that she can now eat well on 19/06/2024 at 11:00am L.A

NURSING CARE PLAN DURING ANTENTAL

DATE/TIME	NURSING DIAGNOSIS	NURSING OBJECTIVE/ CRITERIA OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
22/06/2024 at 10:40am	Increased vaginal discharge related to physiological changes in pregnancy	Client will be able to adjust to the vaginal discharge within 48 hours as evidenced by the client verbalizing that she understands the physiology of increased vaginal discharge in pregnancy	1. Explain the physiology of increased vaginal discharge in pregnancy to client. 2. Educate client on perineal care. 3. Encourage client to report any abnormalities in discharges 4. Educate client to avoid practices like douching and sitting on hot water during pregnancy 5. Encourage client to bath regularly 6. Educate client on drying panties under the sun	1. Physiology of increased vaginal discharge was explained to client 4. Client was educated on perineal care 5. Client was encouraged to report on any abnormality in discharge 4. Education was given to client to avoid practices like douching and sitting on hot water during pregnancy. 5. Client was encouraged to bath regularly.	Goal fully met as evidenced by client verbalized that she understood the physiology of increased vaginal discharge in pregnancy on 24/06/2024 at 11:30am L.A

CHAPTER THREE
INTRAPATAL CARE

3.0 INTRODUCTION

Intrapartal care is the care provided to a woman during labour and delivery. Labour is the process by which the fetus, placenta and membranes are expelled through the birth canal (Myles textbook 17th edition).

This chapter deals with the introduction of labour, admission and management of first, second, third and fourth stage of labour and the immediate and subsequent care of the baby after delivery and care plan.

3.1 ADMISSION AND MANAGEMENT OF LABOUR

ADMISSION

Madam O.F reported to the facility with her first daughter at 4:00 pm on 28th June, 2024 with complains of severe waist pain and lower abdominal pains after she called me around 2:30pm. They were welcomed and made comfortable and I also reassured them of being in safe hands. Her antenatal card was taken and quickly glanced through.

Labour history was taken and she confirmed of having seen ‘show’ around 3:00pm. Her daughter was made to wait at the waiting area.

Madam O.F’s delivery items for delivery were checked and she was taken to the first stage room and permission was sought for head to toe examination to be carried out.

Her vital signs on arrival was recorded as follows;

Temperature	36.4degree Celsius
Pulse	92beats per minute
Respiration	22cycles per minute
Blood pressure	116/73 mmHg
Weight	74 kilogram

3.2 MANAGEMENT OF FIRST STAGE OF LABOUR

At the first stage room she was helped to change into a gown and privacy was provided for her to empty bladder. Urine was straw coloured with 150mls volume and tested for protein, acetone and sugar which were all negative.

Madam O.F was asked to lie on her left side for proper flow of blood and oxygen to the foetus. I washed and dried my hands with clean towel. Head to toe examination was done and no abnormality was detected under the supervision of the midwife in charge. The hair was free from dandruff and lice, it was clean and nicely braided. Her sclera was clear with pink conjunctiva with no discharges found, there were no mouth odour, tooth decay nor dental carries. Her ears were clean with no discharges and the neck without any palpable lymph nodes. The breast was firm on the chest with no scars and the arms were proportionate in length, nails were also short and clean. On her lower extremities, the leg had no varicose veins.

Abdominal examination was conducted through inspection, palpation and auscultation/ on inspection the uterus was globular in shape with no scars. Linea nigra was so visible. Straie gravidarium was absent but there was an evidence of foetal movement. On palpation, the lie was longitudinal and presentation was cephalic, Symphysio-fundal was 36 centimetres, maturity was 38weeks plus 6 days and a descent of 3/5th above the pelvic brim. On auscultation, the foetal heart rate was 138 beats per minute with good volume and rhythm. Contractions were 3 in 10 minutes lasting 33 seconds.

At 4:30pm, permission was sought from her for vaginal examination to be done and she agreed and the procedure was explained to her as well. I made a thorough hand washing with soap and clean water and then wore my sterile gloves. She was asked to assume a lithotomy position with her knees flexed and legs separated. I swabbed the vulva with soaked cotton wool. I used my right hand, picked five sterile cotton wool swabs and dipped them in a savlon solution. I removed the swabs from the antiseptic and dropped one at a time into my left hand, I swabbed the vulva with strokes from upwards downwards starting with labia majora and then the labia

minora. I finally parted the labia minora with my left hand and the vestibule was also swabbed with the right hand with a stroke. I then discarded the used swabs.

The vulva was inspected and no abnormalities were detected such as oedema, varicose veins, scars or warts. The index and middle finger of the gloved right hand were inserted gently into the vagina following the curve of carus. The vagina upon examination was warm, moist and slippery. The cervix was thin, soft and effaced and blunt ischial spines were felt. The cervical dilatation was five centimetres (5cm) with membranes intact and no moulding was present, she was monitored on a partograph.

After the examination, perineal pad was applied to the vulva after it was cleaned. Madam O.F was thanked for her cooperation and all the findings were communicated to her. The findings were also documented in the admission book. She was educated on deep breathing exercise when there is contraction to prevent her from pushing through an undilated cervix. Madam O.F was educated to empty the bladder regularly to promote good uterine contraction aiding descent of the foetal head and dilatation .Client was admitted into a comfortable bed, I then showed her the washroom in case wanted to visit the washroom or bath after delivery, I also showed her the second stage room where she would be sent to deliver. Madam O.F was anxious about the outcome of the delivery so she was reassured that everything will be fine.

PREPARATION FOR BIRTH

A helper was identified, both skilled (staff midwife) and unskilled (Madam O.F's daughter) to assist during labour in case their assistance would be needed. Client was reassured baby would be born soon unto her abdomen to promote bonding. I assisted madam O.F wash her hand and clean and clean her abdomen to prepare for the skin to skin care. Good source of light was made available and the room was lightened already. The second stage room was also prepared by closing all windows and switching off all fans to ensure warmth. The resuscitation tray was made available including the suction device, ambubag and mask, stethoscope, scissors, timer,

gloves, baby's clothes and head covering. All equipment were checked for functioning. Emergency drugs and delivery set were made available. Items for birth preparedness assembled on the delivery trolley were;

TOP SHELF

2 Sterile artery forceps

Cord scissors

Sterile drape

Sterile cotton wool swabs

Cord clamp

Bulb syringe in a bowl of antiseptic solution

Episiotomy tray containing sutures, lignocaine, scissors, syringes and needles

Injection tray containing 10mls oxytocin and vitamin k.

BOTTOM SHELF

Bedpan

Measuring jug

Receiver

Mackintosh

Apron

Boots

Goggles

Face mask

Cot sheets

She was asked on the food she preferred to eat and her daughter was made to buy it for her and it was served immediately to help her get more energy. She was also encouraged to take in more water to keep her hydrated. The foetal heart rate, pulse, contraction and respiration were monitored every 30 minutes , descent of head, blood pressure, and urine output were monitored every four hourly and temperature recorded.

At 6:00pm, contractions were 3 in 10 minutes lasting 45 seconds. Madam O.F complained of waist pains.

Contractions and foetal heart rate were checked every 30 minutes. Madam O.F was seen sweating excessively which was because of the stress of labour, I helped her clean her face and assured her everything will be fine.

At 8:30pm, there was spontaneous rupture of membranes with clear liquor, vaginal examination was done to know the dilatation and os was 9cm dilated, moulding was two plus(++) Blood pressure was 117/66mmHg, pulse was 89 beats per minute, temperature was 36.5degree Celsius, descent of 1/5th, urine volume was 100mls and tested for protein acetone and both proved negative. Contractions were 4 in 10 lasting 42 seconds and fetal heart rate 142bpm.

Madam O.F complained of bearing down and having thigh pains around 9:30pm. Vaginal examination was done again to confirm the second stage of labour. On vaginal examination, cervix was fully dilated (10cm), foetal heart rate was 145 beats per minute. She was asked not to push but breathe through her mouth as she was quickly transferred to the second stage room.

3.3 MANAGEMENT OF SECOND STAGE OF LABOUR

Second stage of labour is the phase between full dilatation of the cervical os and birth of the baby. The already prepared trolley was sent to the bed side after the full dilatation of the cervix was confirmed by the midwife on duty. Madam O.F was asked to assume a lithotomy position. I explained to Madam O.F that the cervix was fully dilated and her baby would soon be delivered. Foetal heart rate was checked again and it was 147bpm.

I put on the protective clothes, washed and dried my hands and put on a pair of sterile gloves.

The vulva and upper thighs were cleaned and draped with a clean sheet. She was informed that the baby will be delivered unto her draped abdomen. A clean perineal pad was applied to the 1anus to maintain a clean delivery field and support the perineum.

Madam O.F was encouraged to push with each contraction and rest in between contractions. As her labour progressed, flexion was aided by putting my two fingers on the head until crowning took place. She was asked to breathe through her mouth and the head was delivered by extension. The eyes were quickly cleaned with a sterile swab from inside out. The neck was felt for cord around it but there was none.

Restitution took place resulting in internal rotation of the shoulders and external rotation of the head which brought out the shoulders in the anterior posterior diameter of the pelvic outlet. With gentle downward traction, the anterior shoulder was delivered and the posterior shoulder by an upward traction. The rest of the body was delivered by lateral flexion onto the mother's abdomen spontaneously at 9:45pm with an alive male infant and the mother was congratulated.

3.4 IMMEDIATE CARE OF THE NEWBORN

Immediately the head of the baby was delivered, the face was wiped with sterile gauze. The rest of the body was delivered onto mother's abdomen to keep baby warm.

The baby was further cleaned and cord clamped 8cm from baby's abdomen and 3cm after first clamp and cut in-between after covering with a sterile gauze to prevent the blood from splashing. Baby was shown to the mother to identify the sex and was placed unto the mother's

abdomen for initiation of breastfeeding and increase bonding between mother and baby. An identification band was placed on baby's wrist bearing the name, sex of baby, mother's name and the time of delivery. Apgar score for the first and fifth minutes were 8/10 and 9/10 respectively.

3.5 MANAGEMENT OF THIRD STAGE OF LABOUR

Third stage of labour is the stage at which the placenta and its membranes are completely delivered and bleeding is controlled.

Madam O.F remained in the lithotomy position with the cut end of the cord placed in a receiver at the perineum to receive the placenta, membranes and blood clots. The fundus was palpated gently to rule out the presence of any undiagnosed second twin but nothing was detected.

Injection oxytocin 10 units was given intramuscularly by my assistant on the middle outer aspect of the left thigh at 9:46pm. My client was cleaned up quickly and gloves used during the delivery were also changed. The cord was reclamped closer to the vulva while the cut end was put inside the receiver. My left hand was placed above the symphysis pubis with my palm facing the fundus while holding the forceps in my right hand and with steady traction on the cord, the foetal surface of the placenta was seen at the vulva.

The cord, placenta and membranes were quickly examined for completeness, placenta was delivered at 9:50pm. The perineum, vulva, vagina and cervix were examined using the twelve o'clock and six o'clock method. There were no cervical, vaginal or perineal tears on inspection. Madam O.F was made to put baby to breast to initiate lactation and bonding. All the delivery instruments were also decontaminated for 10 minutes, washed, dried and sterilized for the next use. The delivery room was tidied up and the gloved hands were dipped in 0.5% chlorine solution by immersion before they were removed and hands washed and dried. Feedback was sent to my client's daughter who accompanied her to the facility. All findings and procedures were documented in the delivery book, client's folder and nurse's notes for reference.

3.6 EXAMINATION OF THE PLACENTA AND MEMBRANES

After delivery of the placenta into a receiver, it was placed on a flat surface under a good source of light. All blood clots were removed from maternal surface into a measuring jug to be estimated. The lobes were examined for completeness and everything was intact with no infarcts. The foetal surface of the placenta was bluish grey in colour with smooth and shiny surface. The cord was inserted centrally. I cleaned thee cut end of the cord with a sterile gauze and examined for any abnormalities. One big vein and two arteries were present in the cord. The amnion was peeled right up to the cord insertion to enable a proper view of the chorion. I held the placenta by the cord and I lifted it up allowing membranes to hang, a hand was spread through the membranes for completeness and everything was intact. Placenta was weighed and measured and findings were documented. All delivery instruments were also decontaminated for 10 minutes, washed, dried and sterilized for the next use. The delivery room was tidied up and the gloved hands were dipped in 0.5% chlorine solution by immersion before they were removed and hands were washed and dried. Feedback was sent to the client's daughter who accompanied her. All findings and procedures were documented in the delivery book, client's folder and nurse's notes for reference.

3.7 MANAGEMENT OF FOURTH STAGE OF LABOUR

Fourth stage of labour is the stage at which the mother and baby are closely monitored and observed for any deviation. It ends after six hours, the first one hour is done in the second stage room and the remaining five hours done in the lying-in-ward. Both mother and baby are monitored for 15 minutes for the first 2 hours, 3o minutes for 1 hour and 1 hour for 3 hours.

Fourth stage was monitored and recorded as;

PREVENTION OF DISESEASES

I washed my hands with soap and under a running water and dried with a clean dry towel.

Tetracycline eye ointment was applied on baby's eyes. Injection vitamin k 1mg was given to prevent bleeding. The umbilical cord was cleaned with methylated spirit and cotton and baby was wrapped in a clean dry cloth to provide warmth.

EXAMINATION OF THE NEWBORN

Madam O.F was assisted to breastfeed baby. I sought for her permission to conduct head to toe examination of the baby in order to rule out any deviation from normal and provide the appropriate intervention. I closed all doors, windows and screened the bed. My hands were washed and gloved. I placed the baby on her back then exposed the baby wholly for quick examination, thereafter kept her warm by covering the non-examining part. The baby looked alert, the cry was normal, respiration was 43 cycles per minute, temperature was 36.1 degree Celsius, foetal heart rate was 134 beats per minute, skin was pink colour and there were no lesions on the skin. On examination of the head, the fontanels were normal and the head circumference measured 35cm, baby's length was 50cm and weight was 3.3kg. The eyes were normally placed with no discharges and nose was normally situated and patent and there was no discharge. There was no false teeth and no tongue tie. The rooting, suckling and swallowing reflexes were present. The neck was normal with no rigidity or swelling while the chest was normal with good rise and fall which depicted good respiration that was noiseless. Also checked the apex beat which recorded 134 beats per minute, her nipples were normally situated. The upper extremities were equal in length and size with normal digits and the nail beds were pink in colour with normal palmer creases. Grasping and Moro reflexes were also present. On palpation, of the abdomen, it was normal with no tenderness or masses. The cord had one big vein and two arteries with no signs of bleeding or infection. The opening of the

penis was normally situated, the penis and anus were inspected for patency and there were no adhesion or discharges. The feet were examined for webbed toe, the axillae, groin and popliteal spaces were also examined for abnormalities. Normal flexions of wrist and ankle joints were also confirmed. I turned the baby on his back with head turned on one side to examine the spine; there were no abnormalities like spinal bifida or missing vertebrae.

VITAL SIGNS OF BABY

Temperature	36.1deegree celcius
Heart rate	134 beats per minute
Respiration	43 cycles per minute
Weight	3.3 kilogram

Other recorded measurements were;

Head circumference	35centimetres
Weight	3.3kilogram
Chest circumference	32centimetres
Full length	50centimetres

The findings were within normal range and were communicated to the mother

MANAGEMENT OF MOTHER

Madam O.F and her baby were monitored critically for the first six hours after delivery. Her vital signs were checked every 15 minutes for the first two hours, 30 minutes for the next one hour and hourly for the last 3 hours and 4 hourly till discharge with findings recorded on the fourth stage chart.

Findings were recorded as follows for the immediate post-delivery.

Vital Signs of Mother

Temperature	36.4 degree celcius
Blood pressure	116/68 mmHg
Pulse	84 beats per minute
Respiration	20 cycles per minute

The symphysio-fundal height measured 17cm and the uterus was well contracted.

She was encouraged to urinate frequently to promote good uterine contraction to avoid postpartum haemorrhage. Lochia was red with moderate flow. I encouraged her to change perineal pad frequently when soaked with blood to prevent puerperal sepsis. The baby's cord was inspected for bleeding and there was no bleeding noticed.

After 1-hour observation on her, she was assisted to the lying in ward for continuous monitoring. Madam O.F was served with warm milo and bread.

SUMMARY OF LABOUR NOTE

Madam O.F had a spontaneous vaginal delivery to a live male child on 28th June 2024 at 9:45pm. Apgar score for the first and fifth minute were 8/10 and 9/10 respectively. Injection oxytocin 10 units was given at 9:46pm. At 9:50pm, the placenta and membranes were completely delivered. Baby's weight was 3.3kg and injection vitamin K was given. The perineum was examined under a bright light and were found intact. Mother and baby were cleaned up and skin to skin was done for 1 hour and breastfeeding was initiated.

DURATION OF OBSERVABLE LABOUR

Stages of labour	Started	Ended
First Stage	4:00pm	9:30pm
Second Stage	9:30pm	9:45pm
Third Stage	9:45pm	9:50pm
Total duration of observable labour	5 hours 50 minutes	

CONDITION OF MOTHER AFTER DELIVERY.

Temperature	36.4degree Celsius
Blood pressure	116/68 mmHg
Pulse	84 beats per minute
Respiration	20 cycles per minute
Condition of perineum	intact
Blood loss	150 millilitres
Symphysio-fundal height	17 centimetres
Mother’s condition	Satisfactory

CONDITION OF THE BABY AFTER BIRTH

Head circumference	35 centimetres
Chest circumference	34 centimetres
Length of the baby	50 centimetres
Sex	Male
Birth weight	3.3 kilogram
Urine	passed
Meconium	passed

APGAR SCORE

Indication	First minute	Fifth minute
Appearance	1	2
Pulse	2	2
Grimace	2	2
Activity	1	1
Respiration	2	2
Total	8/10	9/10

CONDITION OF PLACENTA AND MEMBRANES

Maternal surface	Dark red
Lobes	Intact
Foetal surface	greyish blue in colour
Membrane	complete
Cord insertion	Centrally situated Number
of vessels	Two arteries and one vein
Condition of placenta	Healthy and normal.

3.8 NURSING CARE PLAN ON LABOUR

DATE	PROBLEMS IDENTIFIED
28/06/2024	Madam O.F complained of waist pains
28/06/2024	Madam O.F was anxious of the outcome of the labour.
28/06/2024	Madam O.F complained of painful uterine contraction
28S/06/2024	Madam O.F complained of frequency of micturition
28/06/2024	Madam O.F was seen sweating excessively.

SHORT TERM OBJECTIVES

Madam O.F will be relieved of waist pains after 6 hours.

Madam O.F's anxiety will be laid off in 30 minutes time.

Client will be able to cope with uterine contractions throughout labour

Madam O.F will be able to cope with frequency of micturition throughout labour.

Madam O.F will be relieved of discomfort of excessive sweating within 15 minutes.

LONG TERM OBJECTIVE

Madam O.F will be able to go through all the stages of labour without any complication to both mother and baby.

TABLE 2: LABOUR CARE PLAN

DATE/ TIME	NURSING DIAGNOSIS	CRITERIA NURSING OBJECTIVE/ OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
28/06/2024 at 4:20pm	Altered body comfort (waist pain) related to pressure from the presenting part of the foetus	Madam O.F will be relieved of waist pains after 6 hours by her verbalizing she is relieved of waist pains.	1. Reassure client that she will be relieved off pain after delivery. 2. Explain the physiology of waist pains in labour to client. 3. Give sacral massage to client 4. Advice to adopt any position that makes her comfortable 5. Encourage client to do deep breathing exercise at the peak of the contractions	1. Client was reassured she will be relieved off pain after delivery. 2.Physiology of waist pains in labour explained to client 3. Sacral massage was done for client. 4. She was advised to assume any position that makes her comfortable. 5. Client was encouraged to do deep breathing exercise during contractions.	Goal fully met as client verbalized that she is relieved of waist pains on 28/06/2024 at 10:30pm L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING/ OBJECTIVES/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	EVALUATION /DATE/TIME SIGN
28/06/2024 at 4:40pm	Altered mental state (anxiety) related to unknown outcome of labour	Madam O.F will be relieved of anxiety within 30 minutes as evidenced by client verbalizing that she no longer anxious and can cope with the processes of labour.	1. Reassure client that she is in the hands of a competent team of health workers. 2. Explain to her every procedure to be carried out 3. Give her feedback after every procedure. 4. Introduce her to other mothers who have delivered successfully. 5. Allow client to ask question and answer confidently.	1. Client was reassured of being in the hands of a competent midwife 2. Every procedure to be carried on Madam A.R was explained to her. 3. She was given feedback after every procedure. 4. Client was introduced to other mothers who have delivered successfully 5.Client was allowed to ask questions and they were answered confidently	Goal fully met as evidenced by the client verbalizing that she is no more anxious of the outcome of labour. On 28/06/2024 5:12pm L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/OUTCOME CRITERIA	NURSING ORDERS	NURSINGG INTERVENTION	EVALUATION DATE/TIME/SIGN
28/06/2024 At 5:45pm	Altered body comfort (painful uterine contraction) related to process of labour	Client will be able to cope with labour pain as evidenced by midwife visualizing that client is coping with labour pains 2 Client verbalizing that she has understood what has been taught.	1 Reassure client she will be well 2Explain the cause of labour to client 3 Teach and encourage to practice breathing exercise 4 Sacral massage to the region during contractions. 5.Encourage client to assume a comfortable position during contractions	1 Client was reassured that she will be well. 2 The cause of the labour was explained to client. 3 Client was taught and encouraged to practice breathing exercise. 4 Sacral region massages were done during contractions by midwife. 5.Client was encouraged to assume comfortable position when experiencing contractions	Goal fully met as midwife visualized that client was coped with labour pains and the client verbalized that she understood the physiology of labour pain and the pain has subsided On 28/06/2024 L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	EVALUATION /DATE/TIME SIGN
28/06/2024 At 7:30pm	Impaired comfort (frequency of micturition) Related to pressure of foetal head on the bladder.	Client will be able to cope with frequency of micturition throughout labour as evidenced by client verbalizing that she is coping	1.Reassure client of relief after delivery 2.Explain the physiology of micturition to client 3.Provide clean bedpan at the bed side 4. Educate her on personal hygiene to prevent infections.	1.Client was reassured 2. The physiology was explained to client. 3.A clean bed pan was provided at the client's bedside 4.Client was educated on personal hygiene to prevent infections.	Goal fully met as client verbalized that she is coped with frequency of micturition on 28/06/2024 L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/SIGN
28/06/2024 At 6:30pm	Altered body comfort (excessive sweating) related to stress of labour	1 Client will be relieved of discomfort within 15 minutes as evidenced by client verbalizing that she is relieved 2 Midwife visualizing that client is no more sweating	1. Reassure client that she is going to be relieved 2. Clean client's body and face with wet towel 3. Give sips of water to replace lost fluid. 4.Switch on fan for client	1. Client was reassured that she will be relieved of excessive sweating 2. Client's face and body was wiped with wet towel 3. Sips of water was given to client. 4. Fan was switched on for client.	Goal fully met as midwife visualized client's relaxed facial expression. On 28/06/2024 At 6:45pm L.A

CHAPTER FOUR
POSTNATAL CARE

4.0 INTRODUCTION

This chapter talks about postnatal home visits, postnatal clinic and termination of care.

4.1 DAY OF DELIVERY

After one-hour observation, Madam O.F and her baby's general conditions were reassessed and when they were stable they were transferred to the lying in ward.

I continued to observe them closely to detect any deviation from normal. On abdominal examination, the uterus was firm and well contracted and fundal height measured 17cm above the symphysis pubis. The procedure was explained to her and sought permission to inspect lochia. The lochia was red in colour on inspection and flow was moderate without clots.

She was educated to keep perineum clean and dry by changing the perineal pad as often as possible when it gets soaked. She was also educated to wash her hands before and after changing the pad to prevent germs from being transferred to the vulva which can lead to genital tract infection. I educated her to take in diet rich in fibre, protein and also fruits and vegetables to help regain her health and repair worn out tissues. The high fibre diet is also to improve her bowel movement.

She complained of after pains especially when the baby was suckling the breast. She was reassured and explained to her the physiology of lactation. She was encouraged to empty her bladder frequently and to continue breastfeeding the baby. A warm complex was applied on her lower abdomen and two tablets of paracetamol (1g) were served to relieve her from the pains. Her relatives were informed about her safe delivery and were congratulated for their immersed support. Her vital signs were recorded on the post-delivery chart.

Temperature 36.1 degree Celsius

Blood pressure 117/78mmHg

Pulse 88 beats per minute

Respiration 22 cycles per minute.

Baby was intradermally injected with Bacilli Chalmette Guerin (BCG) and oral polio O vaccine. Client was educated not to apply anything on the site of injection.

Madam O.F was informed that she was to be moved to the lying in ward before she would be discharged.

4.2 SUBSEQUENT CARE OF THE BABY DURING PUERPERIUM.

BABY'S FIRST BATH AND CORD DRESSING

Madam O.F's baby was given a warm bath after 6hours of close observation. Urine and meconium were passed within 24hours of observation. Baby's first bath was given at 7:00am, procedure was explained to the mother and permission was sought from her and she granted me permission. I collected and arranged all items needed for baby bath and set the trolley with the following requirements;

TOP SHELF

Chlorhexidine

Sterile cotton wool swabs.

Cord clamp

Sterile water in galipot

BOTTOM SHELF

Gloves

Mackintosh apron

Jug of water

Baby's soap in a soap dish

Baby's towel and cot sheet

Baby's napkin

Baby's dress

Baby's soft towel and cot sheet

Baby's oil and Vaseline

Receptable for soiled linen

BABY BATH

I assembled all the items and wore a mackintosh apron, mixed the hot water and cold water and tested the temperature of the water with my elbow, went for the baby and placed him on a warm surface. I washed and dried my hands and put on examination gloves. Baby was undressed and covered with a cot sheet. Her eyes were cleaned with a sterile cotton dipped in sterile water cleaning from inner canthus outwards. His face was cleaned with a damp face towel. I took the sponge and soap to lather and the baby's neck was supported with my left hand and my thumb and index fingers were plugged into baby's ears to prevent water from

entering. The hair was washed in a circular motion with a soapy sponge, rinsed and dried with a dry towel and front of the trunk to the feet was washed and turned to the feet and turned the back down to the feet.

Baby's body was immersed in a bath of warm water and soap was rinsed. Baby was lifted gently and from the water and put on a clean dry cot sheet leaving the cord uncovered. I discarded my gloves, washed and dried my hands.

A tray set for dressing of the cord contained;

chlorhexidine

A pair of sterile gloves

Receiver for soiled swabs.

I put on sterile gloves, I applied the chlorhexidine to the base, stump and tip of the cord. Baby was wrapped in a warm cot sheet and was sent to the mother to be breastfed and I thanked her. All linens belonging to the baby was given to the mother and the bath cleaned. I decontaminated all items used in 0.5% chlorine solution for 10 minutes, washed, rinsed and sterilized. All procedures were documented and shown to the midwife in charge.

All findings were communicated to my client, baby was wrapped and put to mother's breast.

The vital signs were taken and recorded as;

Temperature	36.5 degree Celsius
Apex beat	134 beats per minute
Respiration	40 cycles per minute

Baby was wrapped and handed over to the mother and all findings were communicated too her. Mother was thanked and encouraged to maintain good personal hygiene and prevent infection for both herself and the baby.

4.3 FIRST DAY POST DELIVERY AND DISCHARGE

At 6:00 am on 29th June 2024, Madam O.F had a warm bath and was served with porridge and bread. She complained of not been able to sleep due to the feeding the baby at night. I reassured her she will be able to sleep as time goes on and encouraged her to sleep whenever the baby sleeps during the day time. Madam O.F was educated to feed baby on demand. After the baby has slept, head to toe examination of the mother was explained to her. She was asked to empty the bladder.

On head to toe examination, nothing abnormal was detected. Her symphysio fundal height was 16 centimetres. Her perineal pad was inspected and lochia was red with moderate flow and odourless.

Her vital signs were checked and recorded as;

Temperature	36.5 degree Celsius
Blood pressure	115/63mmHg
Pulse	74beats per minute
Respiration	20cycles per minute

Findings were communicated to client and recorded on the mother's chart.

Baby was also examined, he looked alert and the cry was normal with respiration rate of 42 cycles per minute, temperature was 36.3 degree Celsius, foetal heart rate was 132 beats per

minute, head circumference of 35 centimetres with normal fontanels, baby's weight was 3.3kg and a lenth of 50 centimetres. The eyes were normally placed with no discharges, nose was normally situated and patent with no discharges. Cord was clean with no discharges. No abnormality was detected.

Madam O.F's relatives were informed of her discharge and fit to go home.

Client was educated on the importance of rest and sleep and early ambulation for the wellbeing of the mother. I also emphasized about diet and personal hygiene both on mother and baby, exclusive breastfeeding and how to care for the baby.

The following drugs were given;

Tablet Folic Acid 5mg daily for 7 days.

Tablet ferrous sulphate 200mg daily for 7days

Tablet Paracetamol 500mg three times daily for 7days

The client was encouraged to send baby for six days and six weeks postnatal. Finally, I told her to pay attention to baby's cry, sleeping pattern, stool and urine. Her bills were settled and she was informed about the seven days postnatal home visits I will be rendering.

I assisted Madam O.F to pack her things and she was finally discharged around 9:00am.

She thanked the staffs and bid farewell to other clients at the facility.

4.4 FIRST DAY POSTNATAL HOME VISIT

EVENING

On 29th June 2024 at 4:00pm, Madam O.F and family were visited. The whole family were in good condition and mother had already taken her bath. Mother was asked how the family was doing especially the new born and the two daughters whom she said they were all doing well. I asked my client about the after pain and she stated that she is coping.

Madam O.F was examined from head to toe which no abnormality was detected. The breast was lactating and perineal pad was inspected and lochia was rubra and volume was moderate. Baby was top and tailed and cord dressing was done with in the presence of the mother and elder daughter. Cord was inspected for bleeding and any sign of infection but none was detected.

With the mother supporting the baby, I washed and dried my hands and gloves were worn. Inspection of cord was done for wetness and bleeding. I held the tip of the cord clamp and applied chlorhexidine to the base, stump and tip of the cord. Cord was exposed to dry. Gloves used were discarded safely, washed and dried my hands and gave baby to the mother to be breastfed.

Mother was educated not to apply any herbs on the cord to prevent cord sepsis. Client complained of baby vomiting after being breastfed. She was educated on proper positioning and attachment of baby to the breast and I also encouraged her to break wind to wind after each feed. She was also educated on proper perineal hygiene and mild exercise for good muscle tone and drainage of lochia. I informed her about the next visit on the next day and sought permission to leave.

Their vital signs were checked and recorded as;

MOTHER

Temperature 36.2 degree Celsius

Pulse 82 beats per minute

Respiration 22 cycles per minute

Blood pressure 108/75 millimetre per mercury

Fundal height 16 centimetres

Lochia Rubra and moderate

BABY

Temperature 36.4 degree Celsius

Apex beat 133 beats per minute

Respiration 38 cycles per minute

4.5 SECOND DAY POSTNATAL HOME VISIT

MORNING

Madam O.F was visited in her home for the 2nd day postpartum which is the 30th of June 2024 at 7:00am in the morning. The whole family were in good condition and were expecting me because they were told the previous evening that I will be coming to visit them. The house was well organized and I congratulated them for keeping the house neat. I was offered a seat in

their room. I asked about her insomnia and she verbalized she slept well. The procedure that would be carried out on the mother and baby was explained to them. The mother and baby were examined from head to toe and no abnormality was detected. The colour of the baby's stool was brown. Baby was top and tailed and cord was dressed with chlorhexidine and left uncovered to keep it dry.

Client also complained of back pains, she was reassured and was asked to breastfeed baby under my supervision and additional corrections on proper positioning and attachment of baby to the breast was made and advised to change soiled napkin promptly.

Their vital signs were checked and recorded as follows;

MOTHER

Temperature	36.6 degree Celsius
Pulse	72 beats per minute
Respiration	22 cycles per minute
Blood pressure	116/80mmHg
Fundal height	15centimetres

BABY

Temperature	36.8 degree Celsius
Heart rate	136 beats per minute
Respiration	41 cycles per minute

Cord clean and dry

EVENING

Madam O.F and the baby was visited again in the house at 4:00pm the entire family were in good condition. I was offered a seat in the room and I stated the purpose of my visit. Items needed for top and tail were made ready. Mother's perineal pad was inspected and lochia was rubra and not offensive in odour. Madam O.F was reminded on the importance of exclusive breastfeeding. She was educated on kegel exercises to help pelvic floor muscles to gain its muscle tone quickly. She was told to be visited the next day and permission was sought to leave.

Vital signs were checked and recorded as follows;

MOTHER

Temperature 36.4 degrees

Pulse 81 beats per minute

Respiration 21 cycles per minute

Blood pressure 118/60mmHg

Fundal height 15 centimetres

BABY

Temperature 36.6 degree Celsius

Heart rate 141 beats per minute

Respiration 35 cycles per minute

Cord clean and dry

4.6 THIRD DAY POSTNATAL HOME VISIT.

MORNING

Client was visited at home on the 1st of July 2024 in the morning at 7:00am. The aim of the visit was to access the health status of the client and enquire if there has been any improvement in her previous complaint to which she said she has been relieved of the backache. She complained of fatigue as she is not able to rest much. Permission was sought to do head to toe examination on baby which was granted. No abnormalities were detected. General examination was done on the mother and no abnormalities were detected.

Baby was top and tailed and baby's cord was dressed and was healthy. The baby passed stool which was brownish yellow in colour. Head to toe examination was done with no abnormalities were detected. Lochia was moderate and red in colour with no offensive odour. I reminded her of the next visit. I thanked her and sought permission to leave.

Vital signs for both mother and baby were checked and recorded as;

BABY

Temperature 36.5 degree Celsius

Heart rate 138 beats per minute

Respiration 40 cycles per minute

Cord clean and dry

Fundal height 14 centimetres

MOTHER

Temperature 36.7 degree Celsius

Pulse 86 beats per minute

Respiration 22 cycles per minute

Fundal height 15 centimetres

Blood pressure 120/75mmHg

EVENING

Madam O.F and her family were visited at 4:00pm. They were all doing well. Madam O.F verbalized that she slept well. Breast was lactating and lochia was red when her perineal pad was checked. Head to toe examination was done and no abnormalities were detected. Baby was top and tailed. I inspected the cord and it was clean and dry. I dressed the cord with chlorhexidine and wrapped the baby nicely and handed him over to the mother to be breastfed. Client was reminded on the next. I thanked her and sought her permission to leave and permission was granted.

Vital signs were checked and recorded as follows;

MOTHER

Temperature 36.6 degree celsius

Pulse 79 beats per minute

Respiration	20 cycles per minute
Blood pressure	112/74mmHg
BABY	
Temperature	36.2 degree celsius
Heart rate	133 beats per minute
Respiration	37 cycles per minute
Cord	clean and dry

4.7 FOURTH DAY POSTNATAL HOME VISIT

On 2nd July 2024, at 7:30am, I visited my client and the entire family in their home in the evening. Everybody was doing well. Head to toe physical examination was conducted on mother. No abnormalities were detected on both mother and baby. Perineal pad was inspected, lochia was pinkish in colour and scanty in nature without clot and not offensive indicating serosa. Head to toe examination was carried on the baby with no abnormality detected and baby was top and tailed. Baby's cord was clean and dry. I applied chlorhexidine on the cord. Madam O.F said baby has passed urine and stool and the stool was brownish yellow in colour. I checked both mother and baby's vital signs. I asked if she had any complains and she complained of painful breast when breastfeeding so I encouraged her to increase feeding pattern and durations also take in more fluid. I thanked her promised to visit her the next day. I then requested to leave.

MOTHER

Temperature	37.0 degree Celsius
Pulse	77 beats per minute
Respiration	20 cycles per minute
Blood pressure	110/70 mmHg
Fundal height	13 centimetres

BABY

Temperature	36.9 degree Celsius
Heart rate	130 beats per minute
Respiration	37 cycles per minute

4.8 FIFTH DAY POSTNATAL HOME VISIT

Client was visited at home on 3rd July 2024 around 7:00am to check on her and the baby's wellbeing. Mother and baby were in good condition. I applauded the mother for taking good care of herself and baby, I asked about the pain in her breast and she told me the pain has been subsided. I encouraged her to continue the practice and reminded her not to bath baby until cord is off. Madam O.F and baby were examined and nothing abnormal was detected. Baby was top and tailed and cord was dressed. Lochia was serosa on inspection of perineal pad.

Vital signs and other assessments were checked and recorded as;

MOTHER

Temperature	36.6 degree Celsius
Pulse	82 beats per minute
Respiration	21 cycles per minute
Blood pressure	111/78 mmHg
Fundal height	12 centimetres

BABY

Temperature	36.5 degree Celsius
Respiration	43 cycles per minute
Heart rate	134 beats per minute
Stool	passed and yellow in colour
Urine	passed
Cord	clean

4.9 SIXTH DAY POSTNATAL HOME VISIT

Mother and baby were visited in the morning on the sixth day postnatal around 7:00am on

4th July, 2024. Mother had already had her bath and eaten. I asked about her health and that if the family and she verbalized they are all doing well. She also verbalized that she has not been feeling pain in her breast and se can feed her baby well. Physical examination was done on the mother and no abnormalities were found, lochia was pink in colour with moderate flow.

She was educated to wipe baby's buttocks and change napkin immediately it gets soiled. She was also advised to keep baby's clothing separately from hers and dry them under sun rays to prevent skin infection from baby's genital infections. Mother and baby's vital signs were checked and recorded as;

MOTHER

Temperature	36.7 degree Celsius
Pulse	88 beats per minute
Respiration	20 cycles per minute
Blood pressure	105/70 mmHg
Fundal height	12 centimetres

BABY

Temperature	36.6 degree Celsius
Respiration	39 cycles per minute
Heart rate	136 beats per minute
Stool	passed and clear
Urine	passed
Cord	clean

Client was reminded that I will be ending my home visit the next day.

4.10 SEVENTH POSTNATAL HOME VISIT

I visited the client at home around 7:00am in the morning on 5th July, 2024 and met her breastfeeding her baby. I was warmly welcomed and offered seat. I congratulated Madam O.F for correctly attaching the baby to breast I enquired from her if she had any question or complains but she had none. Baby was top and tailed by mother under my supervision and examination was done in the presence of the mother and no abnormality was detected and baby was lactating well. Cord dressing was done by the client under my supervision. Baby was able to suckle well and had passed urine and stool according to mother. Mother was also examined from head to toe and no abnormalities were found. Perineal pad was inspected and lochia was pinkish in colour. Madam O.F was encouraged to continue adhering to all advices given to her especially on nutrition, exercise, rest and sleep and maintaining good personal hygiene and environment. I advised her to take good care of the baby and breastfeed him exclusively. Client was also educated on the importance of immunizing the baby against the vaccine preventable diseases. I expressed my heartfelt gratitude to Madam O.F and her family. I thanked them for their support and cooperation and reminded Madam O.F on her first postnatal visit to the clinic which will be on the 8th of July 2024. I made her aware that I will be waiting for her at the clinic and sought permission to leave.

Their vital signs were checked and recorded as;

MOTHER

Temperature	36.4 degree Celsius
Pulse	80 beats per minute
Respiration	22 cycles per minute
Blood pressure	116/76 mmHg

Fundal height	12 centimetres

BABY

Temperature	36.2 degree Celsius
Respiration	43 cycles per minute
Heart beat	135 cycles per minute

4.11 FIRST POSTNATAL VISIT TO THE CLINIC

On 8th July 2024, Madam O.F and baby accompanied by her husband arrived at the clinic for postnatal care at 8:20am. They were warmly welcomed and given a seat.

Mother and baby were looking cheerful and healthy. She was encouraged to have enough time for the baby to breastfeed for long enough and also change baby's napkins and cot sheets promptly when soiled.

Routine steps and procedures of postnatal clinic were explained to them and invited to the postnatal examination room. Vital signs were checked and recorded as follows for both mother and baby.

MOTHER

Temperature	36.3 degree Celsius
Pulse	86 beats per minute
Blood pressure	116/67 millimetres of mercury
Weight	63 kilogram
Respiration	20 cycles per minute
Haemoglobin level	12.0 gram per decilitre

Urine, protein, sugar	Negative

BABY

Temperature	36.5 degree Celsius
Heart beat	137 beats per minute
Weight	3.4 kilogram
Respiration	36 cycles per minute

Firstly, head to toe examination was done on baby. Findings were given to her alongside examination. The fontanelles were still pulsating, I further informed her that the anterior fontanelle will close at 18 months and the posterior at 6 weeks on its own hence they should not apply hot water or any herbs on the fontanelles since it can cause damage and infection to the brain. She was educated to bring baby to the child welfare clinic for child to be immunized against the childhood killer diseases. The cord was completely healed. Baby was handed to one of the staffs for the mother to be examined as well. Procedure was explained to her, specimen bottle was given to her to collect urine for urinalysis, sugar and protein. Blood sample was taken to check for haemoglobin level. Generally, no abnormality was found. Findings were communicated to her and were thanked. She was counselled on family planning, importance of immunization and prevention of malaria. Mother was advised to visit the registry to do baby's birth certificate. Six weeks postnatal visit was communicated to her. Finally, client was handed over to the midwife in charge and the Public Health Nurses for continuity of care. The baby was circumcised on the postnatal day visit because the cord was off and healed. They were thanked and again informed of my termination of care as I had told them earlier. I was

happy because my care study was successful since client and baby did not have any complication throughout pregnancy, labour and puerperium.

4.12 NURSING CARE PLAN DURING PUERPERIUM.

DATE	PROBLEMS IDENTIFIED
28/06/2024	Madam O.F complained of after pains.
29/06/2024	Madam O.F complained of sleeplessness.
30/06/2024	Madam O.F complained of backache
01/07/2024	Madam O.F complained of fatigue
02/07/2024	Madam O.F complained of breast engorgement.

SHORT TERM OBJECTIVES

1. Client will be relieved of after pains within 48hours.

2. Client will have at sleep in the night within 24 hours.

3. Client will be relieved of backache within 24 hours.

4. Client will be relieved of fatigue within 24hours.

5. Client will be relieved of breast engorgement within 48 hours.

SHORT TERM OBJECTIVE

Madam O.F will go through puerperium successfully without any complication to her and the baby.

TABLE 3: CARE PLAN DURINGPUERPERIUM

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/ SIGN
28/06/2024 At 9:55pm	Altered in body comfort (After pain) related to the release of oxytocin to aid the process of involution.	Client will be relieved of pains within 48hours as evidenced by client verbalizing that pain has decreased	1.Reassure client that pain will decrease with rest and analgesic 2. Educate client on the cause of abdominal pain. 3.Encourage client to continue breastfeeding baby 4.Encourage the client to apply warm compress to the lower abdomen to relieve pain 5.Sever paracetamol (1g) to reduce pain	1. Client was reassured that pain will be relieved with rest and analgesic 2. The cause of pain was explained to client 3. Client was encouraged to continue breastfeeding baby 4. Client was encouraged to apply warm compress to lower abdomen. 5. Tablet paracetamol (1g) was served	Goal fully met as client verbalized that pain has stopped. 30/06/2024 At 10:00am L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	
29/06/2024 At 7:30am	Altered sleeping pattern (sleeplessness) related to feeding baby at night	Client will be relieved of sleeplessness within 24hours as evidenced by client verbalizing shecan sleep well.	1Reassure client that she will be able to sleep at night and 2hours during the day 2.Encourage client to breastfeed baby before she goes to bed 3Advice client to sleep whenever baby sleeps. 4.Encourage client to rest during the day.	1. Client was reassured that will be able to sleep at least 6hours in the night and 2 hours during the day. 2.Client was encouraged to breastfeed baby before she goes to bed 3. Client was advised to sleep whenever baby sleeps. 4.She was encouraged to rest during the day.	Goal fully met as evidenced by client verbalizing that she has been able to sleep well. 01/07/2024 At 7:30am L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/ SIGN
30/06/2024 7:40 am	Impaired comfort (Backache) related to poor posture during breastfeeding	Client will be relieved of backache within 24hours as evidenced by client verbalizing that she is relieved of pain	1.Reassure the client 2. Teach client the proper position to adopt during breastfeeding. 3.Assist client to fix and attach baby to breast. 4. Encourage client to support her back with a pillow while feeding the baby. 5. Educate daughter to assist her in care of the baby for mother to rest.	1. Client was reassured. 2.Client was taught the proper position to adopt during breastfeeding 3. Client was assisted to fix and attach baby to the breast. 4. Client was encouraged to support her back with pillow when sitting 5. Client's daughter was educated to assist in the care of the baby.	Goal fully met as evidenced by client verbalizing that there is no more pain 01/07/2024 At 7:45am L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVES/ CRITERIA OUTCOME	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME SIGN
01/07/2024 At 7:20am	Activity intolerance (fatigue) related to inadequate rest	Client will be relieved of fatigue within 24 hours as evidenced by client verbalizing that she is relieved of fatigue	1.Reassure client that it will resolve with time 2.Explain the need to have enough rest 3. Encourage client to rest when baby is asleep. 4. Encourage relatives to help in household chores 5. Encourage client's husband to help in the care of the baby.	1. Client was reassured that will resolve with time. 2. Client was given an explanation on the need for enough rest 3. Client was encouraged to have enough rest and sleep when baby is asleep 4. Relatives were encouraged to help in household chores. 5. Client's husband was encouraged to help in the care of the baby.	Goal fully met as client verbalized that she is relieved of fatigue on 02/07/2024 At 7:00am L.A

DATE/ TIME	NURSING DIAGNOSIS	NURSING OBJECTIVE/ OUTCOME CRITERIA	NURSING ORDERS	NURSING INTERVENTION	EVALUATION DATE/TIME/ SIGN
02/07/2024 7;00am	Acute pain (breast engorgement) related to infrequent feeding of baby.	Client will be relieved of breast engorgement within 48 hours.	1. Reassure client that she will be able to breastfeed baby without pain. 2. Encourage client to increase feeding pattern and duration. 3. Encourage client to apply warm compress. 4. Encourage client to breastfeed baby frequently. 5. Educate client to gentle use breast pump in severe cases.	1. Client was reassured that she will be able to breastfeed baby without pain. 2. She was encouraged to increase feeding pattern and duration. 3. Client was encouraged to apply warm compress. 4. Client was encouraged to breastfeed baby frequently. 5. Client was educated to gentle use breast pump in severe cases.	Goal fully met as evidenced by client verbalizing that she has relieved of pain in the breast. 04/06/2024 At 7:05am L.A

TERMINATION OF CARE

Explanation was given to Madam O.F on the need to be handed over to the midwife in charge and Community Health Nurse for continuity of care on 9th July, 2024. She was made aware that the program was ending that day, client was reassured of the community health nurse's competency and support. Client was thanked for her cooperation and information provided during the care. Client was officially handed over to the Community Health Nurses and formal introduction was done. She was reassured by the Community Health Nurse that she is in safe hands and readiness to continue caring for her and the baby. The schedule for child welfare clinic and immunization was made known to client. Client was finally thanked as I bid her goodbye and bid her farewell.

SUMMARY AND CONCLUSION

The client and family centered Maternity care study was conducted on Madam O.F, a 36year old Gravida three Para two alive and entire family during antenatal, labour and puerperium. . Client was met on 10th June, 2024 at Nsoatre Health Centered, she was 36 weeks pregnant and in good health. She was under my care through the 36th week of pregnancy, labour and puerperium. Client had a spontaneous vaginal delivery of an alive male baby with the weight of 3.3 kilograms on the 28th of June, 2024, Client was very cooperative and we encountered no complications. They were visited for seven continuous days postpartum.

Mother and baby were managed accordingly and all findings were communicated to mother. Mother was helped to gain more information about exclusive breastfeeding and family planning.

Education was given on diet, breastfeeding, immunization, personal hygiene and waste disposal. She was finally handed over on the first day postnatal clinic in good condition to the midwife in charge and Community Health Nurses team for continuity of care at Nsoatre Health Center.

I complete by saying, the family centered maternity care study has helped me gain more knowledge through practical contact and theoretical ways which I can put into practice and deal with maternity problems as a midwifery professional.

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Table 4: COMPLETE DIAGNOSTIC INVESTIGATION ON THE MOTHER

DATE	SPECIMEN	INVESTIGATION	NORMAL VALUES	FINDINGS	REMARKS
10/11/23	Blood	PMTCT	Negative	Negative	Normal.
	Blood	Haemoglobin level	11-16g/dl	10.8g/dl	Normal
	Blood	Malaria	Negative	Negative	Normal
	Blood	Syphilis	Negative	Negative	Normal
13/12/23	Blood	Blood group	A/B/O/AB	O	Normal.
	Blood	Rhesus factor	positive/negative	Positive	Normal
	Blood	Hepatitis B	negative/positive	Negative	Normal
	Blood	Sickling test	negative	Negative	Normal
12/01/24	Blood	G6PD	No Defect/Full Defect/Partial Defect	No Defect	Normal
	Urine	Protein	Negative/positive	Negative	Normal
			Negative/positive		

	Urine	Sugar		Negative	Normal
19/01/24	stool	Stool RE	Negative/positive	Negative	Normal
09/02/24	Blood Urine Urine	Haemoglobin Protein Sugar	11-16g/dl Negative/positive Negative/positive	10.9gdl Negative Negative	Normal Normal Normal
08/03/24	Blood Urine Urine	Haemoglobin Protein Sugar	11-16g/dl Negative/positive Negative/positive	11.5g/dl Trace Negative	Normal Normal normal
08/04/24	Urine Urine Blood	Protein Sugar Hemoglobin level	Negative/positive Negative /positive 11-16g/dl	Negative Negative 11.5g/dl	Normal Normal Normal
06/05/24	Urine Urine Blood	Protein Sugar Haemoglobin	Negative /positive Negative/positive 11-16g/dl	Trace Negative 11.9g/dl	Normal Normal Normal

DATE	SPECIMEN	INVESTIGATIONS	NORMAL VALUES	FINDINGS	REMARKS
03/06/24	Blood Blood Urine Urine	PMCTC Haemoglobin Protein Sugar	Negative 11-16g/dl Negative /positive Negative /positive	Negative 12.3g/dl Negative Negative	Normal Normal Normal Normal
10/06/24	Urine	Protein Sugar	Negative/positive Negative /positive	Negative Negative	Normal Normal
17/06/24	Urine	Haemoglobin Protein Sugar	11-16g/dl Negative /positive Negative /positive	12.4g/dl Negative Negative	Normal Normal Normal
24/06/24	Urine	Protein Sugar	Negative /positive Negative /positive	Negative Negative	Normal Normal

Table 5: PHAMACOLOGICAL DRUGS FOR THE MOTHER.

DRUG	CLASSIFICATION	DOSAGE	ROUTE	ACTION	ACTUAL EFFECT	SIDE EFFECT	SIDE EXPERIENCE
Tablet ferrous sulphate	Iron supplement	200 milligrams daily for 30 days.	Oral	proper formation and functioning of red blood cells	Haemoglobin level increase	Gastrointestinal disturbances and epigastric pain	None
Tablet multivitamin	vitamin preparation	200 milligram for 30 days	Oral	Increased appetite and helps formation of red blood cells.	increase appetite	Gastrointestinal disturbances.	None
Folic acid	Vitamin preparation	5milligram once daily for 30days	Oral	Helps in the absorption of iron and formation of red blood cells.	increased haemoglobin level	Gastrointestinal disturbances and allergic reaction	None

PHAMACOLOGICAL DRUGS FOR THE MOTHER CONTINUES

DRUGS	CLASSIFICATION	DOSAGE	ROUTE	ACTION/USE	ACTUAL EFFECT	SIDE EFFECT	SIDE EFFECT EXPERIENCED
Tablet sulphadoxinepyrimethamine	Anti-malaria prophylaxis	5 doses. 1st dose starting from 18 weeks or after quickening or and the remaining 4 doses every 4 weeks.	Oral	Prevention of malaria.	Malaria in pregnancy was prevented.	Irritation, nausea and vomiting.	None
Tablet paracetamol	Analgesic and antipyretics	1g tds for 3 times daily x 3days	Oral	relieve pain and reduce increased body temperature	Pain was relieved.	Rare but rashes, blood disorders and liver damage	None

PHAMACOLOGICAL DRUGS FOR THE MOTHER CONTINUED

DRUG	CLASSIFICATION	DOSAGE	ROUTE	ACTION/ USES	ACTUAL EFFECT	SIDE EFFECTS	SIDE EFFECT EXPERIENCED
Tetanol toxoid	Anti – tetanus	0.5 milligram	Subcutaneous tissue	Prevention of tetanus	Prevention of tetanus	Slight Chills	None

PHAMACOLOGICAL DRUGS FOR THE BABY

DRUG		CLASSIFICATION	DOSAGE	ROUTE	ACTION/USE	ACTUAL EFFECT	SIDE EFFECT	SIDE EFFECT EXPERIENCED
Vitamin K		Coagulant	1milligram	intramuscular	Production of prothrombin that aids in clotting	No bleeding	Risk of haemolysis in people with G6PD	None
Gentamycin drop	eye	Prophylaxis Antibiotic	2-3 drops	Instillation	To prevent infection eye	Infection of the eye was prevented	Transient stinging, increased risk of aplastic anaemia	None
Oral polio vaccine		Antigen vaccine	2 drops	Orally	Production o antibodies to prevent poliomyelitis	Poliomyelitis was under observation	There may be diarrhea	None

DRUG	CLASSIFICATION	DOSAGE	ROUTE	ACTION/USE	ACTUAL EFFECT	SIDE EFFECT	SIDE EFFECT EXPERIENCED
Injection Bacillus Calmette Guerin(BCG)	Antigen vaccine	0.05 milligrams	Intradermal	Production of antibody against tuberculosis	Still under observation	Blister formation, Slight fever and pain	None observed

SIGNATORIES

THE PRINCIPAL

NAME: ……

SIGNATURE:……

DATE:……

THE SUPERVISOR

NAME: ……..

SIGNATURE:……

DATE:……..

THE MIDWIFE -IN CHARGE

NAME: …….

SIGNATURE:……

DATE:……

THE STUDENT MIDWIFE

NAME: ……..

SIGNATURE:……

DATE:……